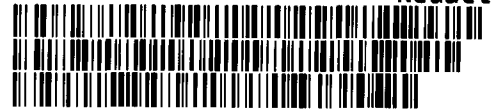


Criteria	Yes	No
Diagnosis Discrepancy		☒
Primary Tumor Site Discrepancy		☒
HIPAA Discrepancy		☒
Prior Malignancy History		☒
Dual/Synchronous Primary		☒
Case is (i/c):	DISQUALIFIED	
Reviewer Initials	Date Reviewed: 4/20/11	

hw 4/25/11

UID: 8C52C92F-26CA-4E72-BE47-CA2ED1746785
 TCGA-AR-A252-01A-PR
 Redacted



Final Diagnosis

Breast, left, wide local excision No. 1: Infiltrating ductal carcinoma, Nottingham grade II (of III) [tubules 3/3, nuclei 2/3, mitoses 1/3; Nottingham score 6/9], forming a mass (1.5 x 1.2 x 1.0 cm) [AJCC pT1c]. Ductal carcinoma in situ, intermediate nuclear grade, solid type comprises approximately 5-25% of the tumor volume. Angiolymphatic invasion is absent. The non-neoplastic breast parenchyma shows proliferative fibrocystic changes. Biopsy site changes are present. The nipple is unremarkable. All surgical resection margins, after a single re-excision of the anterior margin and a single re-excision of the inferior margin, are negative for tumor (minimum tumor free margin, 0.5 cm, anterior inferior margin).

Sentinel lymph nodes; left axillary Nos. 1, 2, and 3; sentinel biopsy: Multiple (4) left axillary sentinel lymph nodes are negative for metastatic carcinoma [AJCC pN0 (i-) (sn)]. Immunohistochemical cytokeratin stain was performed on the paraffin embedded sentinel lymph node tissue and confirms the H&E impression. Blue dye is identified in lymph nodes Nos. 1 and 3. Blue dye is not present in lymph node No. 2.

Lymph node, left axillary, biopsy: A single left axillary lymph node is negative for tumor.

Breast, left, excisional biopsy: Fibroadenoma and previous biopsy site changes. Fibrocystic changes are present. Lateral biopsy margins are negative for tumor.

Seen in consultation with Dr.

ICD-0-3

Carcinoma, infiltrating duct, NOS 8500/3

Site: breast, NOS C50.9
hw
4/25/11